

February 23, 2026

Response from Wolters Kluwer to HHS' Request for Information on  
Accelerating the Adoption and Use of Artificial Intelligence in Clinical Care

Below is Wolters Kluwer's response to the Department of Health and Human Services' Request for Information (RFI) entitled *Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care* (RIN 0955-AA13). We appreciate the opportunity to share our views.

As way of background, Wolters Kluwer is a leading global provider of clinical technology and evidence-based solutions that assist effective decision-making and outcomes across the healthcare continuum. Key solutions include UpToDate®, UpToDate® Lexidrug™, UpToDate® Patient Engagement, Medi-Span®, Sentri7®, Lippincott® Solutions, Ovid®, and Health Language®. Wolters Kluwer had annual revenue in 2024 of \$6.4 billion.

Summary of Our Policy Recommendations

To help accelerate the adoption of artificial intelligence (AI) in clinical care, HHS should define a regulatory policy framework that focuses on the following principles:

- 1) FDA should focus its active regulatory enforcement on AI-enabled medical devices that are fully “agentic,” and impact patient care without human intervention;
- 2) FDA should exercise enforcement discretion for any AI-enabled medical devices featuring ‘human-in-the-loop’ design;
- 3) HHS should define transparency and disclosure standards that are well-understood, predictable and proportionate to substantial risks to patients;
- 4) Existing copyright laws and protections for medical content used to develop AI solutions supporting clinical care should continue to be enforced;
- 5) Existing state medical malpractice laws that seek to optimize the safe practice of medicine should be maintained.

Discussion

In the RFI, HHS notes that it seeks to “establish a regulatory posture on AI that is well understood, predictable, and proportionate to any risks presented to enable rapid innovation while protecting patients and the confidentiality of their identifiable health information and maintaining public trust.” We strongly agree with these overarching goals, particularly in creating a regulatory framework that is predictable and easy to understand for developers, and fosters trust among clinicians and medical professionals.

To strike the appropriate balance between rewarding innovation for AI solutions while also ensuring such solutions are effective and safe for patients, we recommend FDA focus its active regulatory oversight on AI-enabled device software functions that pose the highest risk to patient safety. In our view, these are functions that are fully “**agentic**,” meaning they can act autonomously, making care decisions and/or acting without human intervention.

Though the deployment of agentic functions can add significant value and efficiency to the clinical workflow, freeing up trained human professionals to address more complex patients, they also pose significant threats to patient safety if the software does not perform as designed. Remaining AI-enabled device software functions that are non-agentic, and thus incapable of acting independently of human intervention, would still technically remain under FDA's regulatory authority, but be subject to enforcement discretion. FDA could provide guidance that defines the terms of such discretion. The software's design would require a trained human intermediary to make a clinical decision based on their experience and judgment, with the software itself having no agentic capability.

We were gratified to hear FDA Commissioner Marty Makary, in addressing the Consumer Electronics Show in early January, appear to endorse such an approach. In his remarks, Dr. Makary indicated that the Agency's focus going forward will be to "apply regulatory standards" to AI-enabled solutions that are "being used to trigger, automatically, medical interventions." Focusing FDA on AI-enabled functions that are fully agentic and autonomous also aligns with the Agency's historical regulation of medical devices that operate independent of human intervention.

Dr. Makary also emphasized the need for FDA to avoid paternalism and "trust clinicians to do what's right for their patients." Underscoring the importance of human-in-the-loop design, the Commissioner added that FDA should trust doctors to use what information they find useful in making their medical decision, for which they bear ultimate responsibility.

Simplifying the regulatory framework is only part of the challenge. Clinicians and other medical professionals will be the principal actors in driving adoption of AI tools at the point of care, but it has been our experience that they will not use solutions they do not trust. Such trust can only be earned with appropriate transparency and disclosure concerning the AI-enabled solution.

The need for trust was starkly illustrated by a [survey](#) conducted in late 2024 by Wolters Kluwer, which found that 40% of the U.S. physicians polled were ready to use generative AI (GenAI) when interacting with patients at the point-of-care. The findings reflected a rapid acceptance of the new technology more broadly, with 68% saying they had recently changed their views and were more likely to think that GenAI would be beneficial to healthcare.

However, the survey also revealed that physicians will not blindly follow GenAI recommendations and that they would be selective when evaluating and implementing GenAI tools. For example, before relying on GenAI tools for assistance in clinical decision-making, 91% of respondents indicated they would need to know that GenAI-sourced materials were created by doctors and medical experts. Similarly, 89% reported they would require vendors to be transparent about where information came from, who created it, and how it was sourced. If these survey results are indicative of the wider provider community, and we believe they are,

clinicians will not use GenAI-enabled tools at the point of care unless they understand how such solutions were developed and trust their output.

At the time of this writing, HHS is sending conflicting signals about the types of disclosures it will require AI developers to make, with different requirements being issued or proposed by FDA and the Assistant Secretary for Technology Policy. For example, the draft HTI-5 rule issued in December proposes to scale back and align disclosures for evidence-based and predictive decision support interventions, with many of the latter considered regulated devices. Other FDA guidance spells out labeling recommendations for AI-enabled device software functions, but the Agency has yet to specify similar recommendations for devices using GenAI. The recently revised Final Guidance on *Clinical Decision Support Software* provides labeling recommendations for certain CDS solutions that will be given enforcement discretion, some of which may be enabled by AI.

Given this, we recommend HHS provide clarity as to the types of information that developers of AI solutions used at the point of care should disclose to medical professionals and end users. At a minimum, clinicians should have access to information that explains the provenance of any care recommendations, how such recommendations were developed, and by whom, and any substantial risks to patients associated with relying on the recommendation.

Our responses to specific RFI questions follow:

- 1) *What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.*

We previously commented to HHS and FDA on the Agency's September 2022 Final Guidance entitled *Clinical Decision Support Software* (FDA-2017-D-6569), which provided FDA's interpretation of statutory language included in the 21<sup>st</sup> Century Cures Act of 2016 that exempted low-risk CDS software from FDA's regulatory scope.

We commend FDA for recently issuing a revised Final Guidance granting enforcement discretion to decision support software that issues a single clinically appropriate care recommendation. We also strongly support the Agency's view that the level of software automation is a critical factor that should be considered in determining whether the health care professional has had sufficient opportunity to independently review the basis for the care recommendation.

However, we continue to be concerned that FDA will consider the time-critical nature of medical decision as a factor to determine whether a software solution should be regulated. As we have pointed out in previous comments, the 21<sup>st</sup> Century Cures Act does not authorize FDA to consider time-sensitivity in medical decision-making as a criterion in defining which CDS software would remain under the Agency's regulatory purview. Rather, Congress clearly indicated that the developer's design or "intended use" of the software should control.

Moreover, in practice, identifying time-critical decisions is not as clear as it might seem due to numerous factors, including the patient, their presentation, and the setting in which treatment is being delivered. For example, the management of a patient with gastrointestinal bleeding may be emergent if the patient is severely bleeding and is in shock, whereas it may be less urgent if the patient had a single episode of bleeding an hour earlier and is hemodynamically stable. Thus, given the context-specific nature of time-critical decisions, we believe its use as a factor to determine which CDS software should be regulated is impracticable, unfeasible, and beyond the scope of FDA's statutory authority.

While FDA's recent clarification on CDS that provides a single recommendation is welcome, we fear FDA's continued use of time-criticality will continue to stifle new products and deprive providers and patients of enhanced CDS solutions that can improve health outcomes and lower costs.

We therefore recommend FDA adopt a simpler regulatory approach to CDS software that focuses solely on the software's level of automation, with CDS that is designed to trigger care interventions without a human intermediary being subject to FDA oversight. We note this focus on automation aligns with our earlier recommendation that FDA's regulatory oversight of all AI-enabled devices should focus on software that is "agentic," capable of acting without human intervention.

- 2) *For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?*

While we appreciate HHS' implied distinction in the question between medical devices and non-medical devices, our response addresses two important legal issues that apply to both.

Though not within HHS' regulatory purview, we feel compelled to raise the issue of **copyright protection** and the continued enforcement of federal copyright laws. Copyright and AI are commonly discussed in the context of the creative arts, but it also applies to clinical content such as care guidelines, published medical research or patient information leaflets. Such material can be used by AI developers to train and/or test AI-enabled software solutions used in clinical care to gauge alignment with evidence-based care. Per existing law, before developers can use copyrighted content for creating their AI solutions, permission must first be obtained from the content creator.

Enforcement of existing copyright protections for clinical content should be maintained. Such protections enable clinical content creators to maintain a single, authoritative version, preventing unauthorized edits, distortions or selective quoting. Even small alterations to medical content can introduce clinical risk, misinterpretation or error, putting patients at risk. Empowering the authors of clinical guidelines to control updated versions ensures the continued scientific and medical integrity of the content.

Producing scientifically grounded medical research, practice guidelines or patient information leaflets also requires significant upfront investment in time, resources and costs. Copyright protection not only enables creators to recoup their upfront costs, it encourages future investment in new research, and in follow-on content that applies such research and findings to day-to-day clinical care.

The second legal issue is the call by some that **clinicians be given a shield from legal liability if a patient was harmed when AI was used in their care**. We strongly disagree. Medicare professionals must continue to be responsible for the care they provide. Shielding them from such responsibility could put patient safety at risk, sow distrust among patients and undermine the professional's duty of care. HHS should therefore continue to support existing state tort laws that promote the safe practice of medicine, and clinicians, hospital and health systems using AI-enabled software to assist in clinical decision-making should continue to be subject to state medical malpractice laws.

We appreciate the opportunity to comment. If HHS staff have questions or would like to discuss our responses in more detail, please contact Bob Hussey at [bob@bobhussey.com](mailto:bob@bobhussey.com) who can arrange a discussion with the appropriate Wolters Kluwer staff.